A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

15 April 2026

Dr Sally Cockburn
Junction Place Medical Centre

Dear Dr Cockburn,

Re: Mrs Alida Reggars
DOB: 1947-08-13
Address: 304/619 Canterbury Road, SURREY HILLS VIC 3127

Summary

- **Two-month history of diarrhoea and weight loss in the context of scleroderma.**
- **For investigation of Clostridioides difficile infection and small intestinal bacterial overgrowth.**
- **Dose escalation of nortriptyline and loperamide for symptom control.**
- **Recommendation to defer further rituximab therapy until gastrointestinal symptoms are stabilised.**
- **No action required by you.**

I had the pleasure of conducting a review with Alida. As you are aware, Alida has a complex medical background including scleroderma for which she is immunosuppressed with rituximab. She presented today with a two-month history of persistent diarrhoea and associated weight loss.

We discussed her symptoms in detail, noting that while she previously had well-controlled bowel function, this recent exacerbation began following a course of antibiotics in January. Although she has undergone some preliminary investigations, I noted that a stool specimen for *Clostridioides difficile* has not yet been performed. Given her scleroderma and immunosuppressed status, she is at a higher risk for both antibiotic-associated diarrhoea and small intestinal bacterial overgrowth.

Alida has been attempting to manage her symptoms with loperamide and buscopan. While the buscopan provides some relief for abdominal pain, it has not significantly altered her bowel frequency or consistency. She has also been continuing her maintenance dose of nortriptyline.

All appointments and enquires
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Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
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Impression and Plan My clinical impression is that we must urgently exclude *Clostridioides difficile* infection and investigate the possibility of bacterial overgrowth. I have provided Alida with pathology referrals for a comprehensive stool analysis and blood tests. Regarding her symptomatic management, I have advised her to increase her loperamide dose up to a maximum of eight tablets daily as required to reduce urgency and frequency. I have also increased her nortriptyline dose to 37.5 mg daily to assist with gut motility.

I have suggested to Alida that she should consider deferring her next rituximab treatment with her rheumatologist, Laura Ross, until we have successfully identified the cause of her diarrhoea and achieved clinical stability. I will keep you updated on her progress and have arranged to review her in one month to assess her response to these adjustments and discuss her test results.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Laura Ross

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